xxxxxxxx-5253

For the use of a Registered Medical Practitioner or a Hospital or a Laboratory only

VALPARIN XR 300 MG TABLET

(Controlled release tablets of Sodium Valproate and Valproic Acid)

COMPOSITION

VALPARIN XR 300 MG TABLET

Each film coated controlled release tablet contains:

Sodium Valproate	199.8 mg
Valproic acid	87 mg

(Both together correspond to Sodium Valproate 300mg)

DESCRIPTION

Valparin XR 300 mg Tablet is a broad spectrum antiepileptic drug containing sodium valproate in a Controlled Release Form.

VALPARIN XR 300 mg TABLET: White, round, biconvex film coated tablets.

CLINICAL PHARMACOLOGY

Valproate exerts its effect primarily on the central nervous system. Its anticonvulsant properties are exerted against a wide variety of epilepsies experimental animal models and humans. Its main mechanism of action seems to
be related to reinforcement of the gabaergic pathway.

The various pharmacokinetic studies conducted with valproate demonstrate the following points, some of which may have practical consequences on prescription:

- The blood bioavailability of valproate following oral administration approaches 100 %.
- The volume of distribution is primarily limited to the blood and to rapid-exchange extracellular fluids. Valproate diffuses into the C.S.F. and brain.
- Half-life is approximately 8 to 20 hours and is usually shorter in children.
- Significant therapeutic results are achieved with serum concentrations between 40 to 100 mg / l, but there is no definite correlation between serum level and therapeutic response.
- The steady state plasma concentration is achieved rapidly (3 to 4 days).
- Valproate is extensively bound to proteins. Binding is dose-dependent and saturable.
- Valproate is primarily excreted in the urine following glucuronic acid conjugation and beta- oxidation.
- Valproate molecule is dialyzable, but hemodialysis only affects the free fraction of valproate in the blood (approximately 10 percent).
- Valproate does not induce enzymes involved in cytochrome P 450 metabolism: in contrast to most other antiepileptics, it therefore does not accelerate its own degradation or that of other substances such as oral contraceptives and oral anticoagulants.

In comparison to the enteric form, the slow-release form sodium valproate is characterized at equal doses by:

- Disappearance of the absorption latency time;
- Prolonged absorption;
- Similar bioavailability,
- lowering of maximum plasma concentrations (Cmax) of the total and free fractions (Cmax decreased by approximately 25 %, but relatively stable in a plateau, between the 4th and 14th hour), this flattening of the peaks makes it possible to achieve more even valproic acid concentrations which are more evenly distributed over the 24-hour cycle: Following twice daily administration of the same dose, the amplitude in plasma fluctuations is reduced by half.
- More linear correlation between the dose and plasma concentration (total and free fractions).

INDICATIONS

Treatment of generalized or partial epilepsy, particularly with the following patterns of seizures: Absence, myoclonic, tonic-clonic, atonic and mixed.

For partial epilepsy: Simple or complex seizures, secondary generalized seizures and specific syndromes (West, Lennox-Gastaut).

Treatment and prevention of mania associated with bipolar disorder.

CONTRAINDICATIONS

Acute hepatitis; chronic hepatitis; family history of severe hepatitis; notably drug-induced hepatitis; porpyria; hypersensitivity to sodium valproate.

Sodium valproate is contraindicated in the following situations:

- In epilepsy
- sodium valproate is contraindicated in pregnancy unless there is no suitable alternative treatment
- sodium valproate is contraindicated in women of childbearing potential, unless the conditions of Pregnancy Prevention Programme are fulfilled (see Warnings and precautions and Fertility, pregnancy and lactation sections)
- In bipolar disorder
- sodium valproate is contraindicated in pregnancy
- sodium valproate is contraindicated in women of childbearing potential, unless the conditions of Pregnancy Prevention Programme are fulfilled (see Female children, Women of childbearing potential, pregnant women section)

PRECAUTIONS

Potential for an increase in risk of suicidal thought or behaviours.

CASES OF LIFE-THREATENING PANCREATITIS HAVE BEEN REPORTED IN BOTH CHILDREN AND ADULTS RECEIVING VALPROATE. SOME OF THE CASES HAVE BEEN DESCRIBED AS HEMORRHAGIC WITH A RAPID PROGRESSION FROM INITIAL SYMPTOMS TO DEATH. CASES HAVE BEEN REPORTED SHORTLY AFTER INITIAL USE AS WELL AS AFTER SEVERAL YEARS OF USE. PATIENTS AND GUARDIANS SHOULD BE WARNED THAT ABDOMINAL PAIN, NAUSEA, VOMITING AND/OR ANOREXIA CAN BE SYMPTOMS OF PANCREATITIS THAT REQUIRE PROMPT MEDICAL EVALUATION. IF PANCREATITIS IS DIAGNOSED, VALPROATE SHOULD BE DISCONTINUED.

Liver damage resulting in fatalities has been exceptionally reported. Patients most at-risk, especially in cases of multiple anti-convulsant therapies, are infants and young children under the age of 3 with severe seizure disorders. After the age of 3 the incidence reduces significantly and progressively decreases with age.

Monotherapy is recommended in children under 3 yrs. of age, but the potential benefit of Valparin XR 300 mg Tablet should be weighed against the risk of liver damage. Liver function tests are required before beginning treatment (see contraindications) as is periodic monitoring during the first 6 months, especially in
at-risk patients.

It should be underlined that like most antiepileptic, there may be an isolated and transient increase in liver enzymes, in the absence of any clinical signs especially
at the beginning of treatment. In such cases, a complete laboratory workup should be performed (including prothrombin time), dosage may be reconsidered an workups should be repeated on the basis of the changes in the parameters.

In children, under 3 yrs of age salicylates should not be prescribed concurrently due to the risk of liver toxicity.

In patients with renal insufficiency, increased serum concentrations of free valproic acid should be taken into account, and dosage should be consequently be adjusted.

In case of acute abdominal pain, serum amylase level should be determined before deciding on surgery, as exceptional cases of pancreatitis have been reported.

Blood tests including cell count, platelet count, bleeding time and coagulation tests are recommended prior to initiation of therapy and surgery, and in case of spontaneous bruising or bleeding.

Immune disorders have been noted only exceptionally and hence benefit Valparin XR 300 mg Tablet should be weighed against potential risk in patients with SLE.

Female children/Female adolescents/Women of childbearing potential/Pregnancy

Valparin XR 300 mg Tablet should not be used in female children, in female adolescents, in women of childbearing potential and pregnant women unless alternative treatments are ineffective or not tolerated because of its high teratogenic potential and risk of developmental disorders in infants exposed in utero to valproate. The benefit and risk should be carefully reconsidered at regular treatment reviews, at puberty and urgently when a woman of childbearing potential treated with Valparin XR 300 mg Tablet plans a pregnancy or if she becomes pregnant.Women of childbearing potential must use effective contraception during treatment and be informed of the risks associated with the use of Valparin XR 300 mg Tablet during pregnancy (See Fertility, Pregnancy and Lactation).

Female children and women of childbearing potential

Sodium valproate should not be used in female children and women of childbearing potential unless other treatments are ineffective or not tolerated (see Contraindications, Warnings and precautions and Fertility, pregnancy and lactation sections). The benefit and risk should be carefully reconsidered at regular treatment reviews. Sodium valproate should preferably be prescribed as monotherapy and at the lowest effective dose, if possible as a prolonged release formulation. The daily dose should be divided into at least two single doses.

Sodium valproate has a high teratogenic potential and children exposed in utero to sodium valproate have a high risk for congenital malformations and neurodevelopmental disorders.

Sodium valproate is contraindicated in the following situations:

- In pregnancy unless there is no suitable alternative treatment for epilepsy indication.
- In pregnancy for bipolar disorder indication.
- In women of childbearing potential unless below conditions are fulfilled.

Conditions of Pregnancy Prevention Programme:

The prescriber must ensure that:

- Individual circumstances should be evaluated in each case, involving the patient in the discussion to guarantee her engagement, discuss therapeutic options and ensure her understanding of the risks and the measures needed to minimise the risks.
- The potential for pregnancy is assessed for all female patients.
- The patient has understood and acknowledged the risks of congenital malformations and neurodevelopmental disorders including the magnitude of these risks for children exposed to sodium valproate in utero.
- The patient understands the need to undergo pregnancy testing prior to initiation of treatment and during treatment, as needed.

In women planning to become pregnant all efforts should be made to switch to appropriate alternative treatment prior to conception, if possible (see Fertility, Pregnancy and Lactation).

Sodium valproate therapy should only be continued after a reassessment of the benefits and risks of the treatment with sodium valproate for the patient by a physician experienced in the management of epilepsy.

- The patient is counselled regarding contraception, and that the patient is capable of complying with the need to use effective contraception (for further details please refer to subsection contraception of this boxed warning), without interruption during the entire duration of treatment with sodium valproate.
- The patient understands the need for regular (at least annual) review of treatment by a prescriber experienced in the management of epilepsy.
- The patient understands the need to consult her physician as soon as she is planning pregnancy to ensure timely discussion and switching to alternative treatment options prior to conception and before contraception is discontinued.
- The patient understands the need to urgently consult her physician in case of pregnancy.
- The patient has received the Patient Guide.
- The patient has acknowledged that she has understood the hazards and necessary precautions associated with sodium valproate use (Annual Risk Acknowledgement Form).

These conditions also concern women who are not currently sexually active unless the prescriber considers that there are compelling reasons to indicate that there is no risk of pregnancy.

Female children

The prescriber must ensure that:

- The parents/caregivers of female children understand the need to contact the doctor once the female child using sodium valproate experiences menarche.
- The parents/caregivers of female children who have experienced menarche are provided with comprehensive information about the risks of congenital malformations and neurodevelopmental disorders including the magnitude of these risks for children exposed to sodium valproate in utero.

In patients who have experienced menarche, the prescriber must annually reassess the need for sodium valproate therapy and consider alternative treatment options. If sodium valproate is the only suitable treatment, the need for using effective
contraception and all other conditions of the pregnancy prevention programme should be discussed. Every effort should be made by the prescriber to switch female children to alternative treatment before they reach adulthood.

Pregnancy test

Pregnancy must be excluded before start of treatment with sodium valproate. Treatment with sodium valproate must not be initiated in women of childbearing potential without a negative pregnancy test (plasma pregnancy test) result, confirmed by a healthcare provider, to rule out unintended use in pregnancy.

Contraception

Women of childbearing potential who are prescribed sodium valproate must use effective contraception without interruption during the entire duration of treatment with sodium valproate.

These patients must be provided with comprehensive information on pregnancy prevention and should be referred for contraceptive advice if they are not using effective contraception. At least one effective method of contraception (preferably a user independent form such as an intra-uterine device or implant) or two complementary forms of contraception including a barrier method should be used. Individual circumstances should be evaluated in each case when choosing the contraception method, involving the patient in the discussion to guarantee her engagement and compliance with the chosen measures. Even if she has amenorrhea she must follow all the advice on effective contraception.

Annual treatment reviews by the prescriber

The prescriber should review at least annually whether sodium valproate is the most suitable treatment for the patient. The prescriber should discuss the Annual Risk Acknowledgement Form at initiation and during each annual review and ensure that the patient has understood its content

Pregnancy planning

If a woman is planning to become pregnant, a prescriber experienced in the management of epilepsy must reassess sodium valproate therapy and consider alternative treatment options. Every effort should be made to switch to appropriate alternative treatment prior to conception and before contraception is discontinued. If switching is not possible, the woman should receive further counselling regarding the risks of sodium valproate for the unborn child to support her informed decision-making regarding family planning.

In case of pregnancy

If a woman using sodium valproate becomes pregnant, she must be immediately referred to a doctor to re-evaluate treatment with sodium valproate and consider alternative treatment options. The patients with sodium valproate-exposed
pregnancy and their partners should be referred to a doctor experienced in prenatal medicine for evaluation and counselling regarding the exposed pregnancy.

Pharmacists must ensure that:

- The Patient Card is provided with every sodium valproate dispensation and that patients understand its conten
- Patients are advised not to stop sodium valproate medication and to immediately contact the prescriber in case of planned or suspected pregnancy

Educational materials

In order to assist healthcare professionals and patients in avoiding exposure to sodium valproate during pregnancy, the Marketing Authorisation Holder has provided educational materials to reinforce the warnings, provide guidance regarding use of sodium valproate in women of childbearing potential and provide details of the Pregnancy Prevention Programme. A Patient Guide and Patient Card should beprovided to all women of childbearing potential using sodium valproate.

An Annual Risk Acknowledgement Form needs to be used at time of treatment initiation and during each annual review of sodium valproate treatment by the prescriber. at treatment initiation, at the annual visit, and when a woman plans a
pregnancy or is pregnant

Sodium valproate therapy should only be continued after a reassessment of the benefits and risks of the treatment with sodium valproate for the patient by a doctor experienced in the management of epilepsy.

The prescriber must ensure that the patient is provided with comprehensive information on the risks alongside relevant materials, such as a patient information booklet, to support her understanding of the risks.

In particular, the prescriber must ensure the patient understands:

- The nature and the magnitude of the risks of exposure during pregnancy, in particular the teratogenic risks and the risks of developmental disorders.
- The need to use effective contraception.
- The need for regular review of treatment.
- The need to rapidly consult her physician if she is thinking of becoming pregnant or there is a possibility of pregnancy.

In women planning to become pregnant all efforts should be made to switch to appropriate alternative treatment prior to conception, if possible:

Valproate therapy should only be continued after a reassessment of the benefits and risks of the treatment with valproate for the patient by a physician experienced in the management of epilepsy.

FERTILITY, PREGNANCY AND LACTATION

Valparin XR 300 mg Tablet should not be used in female children, in female adolescents, in women of childbearing potential and in pregnant women unless other treatments are ineffective or not tolerated.

Women of childbearing potential have to use effective contraception during treatment. In women planning to become pregnant all efforts should be made to switch to appropriate alternative treatment prior to conception, if possible.

- Sodium valproate is contraindicated as treatment for epilepsy during pregnancy unless there is no suitable alternative to treat epilepsy.
- Sodium valproate is contraindicated as treatment for bipolar disorder during pregnancy.
- Sodium valproate is contraindicated for use in women of childbearing potential unless the above-mentioned conditions of Pregnancy Prevention Programme are fulfilled (see Contraindications and Warnings and precautions sections)

Pregnancy Exposure Risk related to valproate

Both valproate monotherapy and valproate polytherapy are associated with abnormal pregnancy outcomes. Available data suggest · that antiepileptic polytherapy including valproate is associated with a greater risk of congenital malformations than valproate monotherapy.

Congenital malformations

Data derived from a meta-analysis (including registries and cohort studies) has shown that 10.73% of children of epileptic women exposed to valproate monotherapy during pregnancy suffer from congenital malformations (95% CI: 8.16 -13.29). This is a greater risk of major malformations than for the general population, for whom the risk is about 2-3%. The risk is dose dependent but a threshold dose below which no risk exists cannot be established. Available data show an increased incidence of minor and major malformations. The most common types of malformations include neural tube defects, facial dysmorphism, cleft lip and palate, craniostenosis, cardiac, renal and urogenital defects, limb defects (including bilateral aplasia of the radius), and multiple anomalies involving various body systems.

Developmental disorders

Data have shown that exposure to valproate in utero can have adverse effects on mental and physical development of the exposed children. The risk seems to be dose-dependent but a threshold dose below which no risk exists, cannot be established based on available data. The exact gestational period of risk for these effects is uncertain and the possibility of a risk throughout the entire pregnancy cannot be excluded.

Studies in preschool children exposed in utero to valproate show that up to 30-40% experience delays in their early development such as talking and walking later, lower intellectual abilities, poor language skills (speaking and understanding) and memory problems.

Intelligence quotient (IQ) measured in school aged children (age 6) with a history of valproate exposure in utero was on average 7-10 points lower than those children exposed to other antiepileptics. Although the role of confounding factors cannot be excluded, there is evidence in children exposed to valproate that the risk of intellectual impairment may be independent from maternal IQ.

There are limited data on the long term outcomes.

Available data show that children exposed to valproate in utero are at increased risk of autistic spectrum disorder (approximately three-fold) and childhood autism (approximately five-fold) compared with the general study population.

Limited data suggests that children exposed to valproate in utero may be more likely to develop symptoms of attention deficit/hyperactivity disorder (ADHD).Female children, female adolescents and women of childbearing potential (see above and Special Warnings and Precautions for use)'

If a Woman wants to plan a Pregnancy

- During pregnancy, maternal tonic clonic seizures and status epilepticus with hypoxia may carry a particular risk of death for the mother and the unborn child.
- In women planning to become pregnant or who are pregnant, valproate therapy should be reassessed
- In women planning to become pregnant all efforts should be made to switch to appropriate alternative treatment prior to conception, if possible.

Valproate therapy should not be discontinued without a reassessment of the benefits and risks of the treatment with valproate for the patient by a physician experienced in the management of epilepsy. If based on a careful evaluation of the risks and the benefits valproate treatment is continued during the pregnancy.

Pregnant women

Sodium valproate as treatment for epilepsy is contraindicated in pregnancy unless there is no suitable alternative treatment. If a woman using sodium valproate becomes pregnant, she must be immediately referred to a doctor to consider alternative treatment options.

During pregnancy, maternal tonic clonic seizures and status epilepticus with hypoxia may carry a particular risk of death for the mother and the unborn child. If in exceptional circumstances, despite the known risks of sodium valproate in pregnancy and after careful consideration of alternative treatment, a pregnant woman must receive sodium valproate for epilepsy.

it is recommended to:

- Use the lowest effective dose and divide the daily dose valproate into several small doses to be taken throughout the day.
- The use of a prolonged release formulation may be preferable to other treatment formulations in order to avoid high peak plasma concentrations.
- Folate supplementation before the pregnancy may decrease the risk of neural tube defects common to all pregnancies. However, the available evidence does not suggest it prevents the birth defects or malformations due to valproate exposure.
- To institute specialized prenatal monitoring in order to detect the possible occurrence of neural tube defects or other malformations.

Fertility

Valproate administration may also impair fertility in men (see Section Adverse Reactions). In the few cases in which valproate was switched/discontinued or the daily dose reduced, the decrease in male fertility potential was reported as reversible in most but not all cases, and successful conceptions have also been observed.

ADVERSE REACTIONS

Congenital and familial/genetic disorders: (See Pregnancy and Lactation)

Hepatobiliary disorders:

Rare cases of liver dysfunction, Severe Liver damage, including hepatic failure sometimes resulting in death, has been reported; Increased Liver enzymes are common, particularly early in treatment and may be transient.

Gastrointestinal disorders:

Nausea, gastralgia, diarrhoea frequently occur at the start of treatment. These problems can usually be overcome by taking Valparin XR 300 mg Tablet with or after food.

Very rare case of pancreatitis sometime lethal has been reported.

Reproductive system and breast disorders:

Rare: male infertility

Nervous system disorder:

Sedation has been reported occasionally. usually when in combination with other anticonvulsants. In mono- therapy it occured early in treatment on rare occasions and is usually transient. Rare cases of lethargy and confusion occasionally progressing to stupor, sometime with associated hallucinations or convulsions have been reported. Encephalopathy and coma have very rarely been observed.

These cases have often been associated with too high a starting dose or too rapid a dose escalation or concomitant use of other anticonvulsants, notably phenobarbitone. They have usually been reversible on withdrawal of treatment or
reduction of dosage.

Very rare cases of reversible extrapyramidal symptoms including parkinsonism or reversible dementia associated with reversible cerebral atrophy have been reported

Dose-related ataxia and fine postural tremor have occasionally been reported.

An increase in alertness may occur, this is generally beneficial but occasionally aggression, hyperactivity and behavioural deterioration have been reported.

Metabolic disorders:

Cases of isolated and moderate hyperammonaemia without change in liver function tests may occur frequently are usually transient and should not cause treatment discontinuation. However, they may present clinically as vomiting.ataxia, and increasing clouding of consciousness. Should these symptoms occur Valparin XR 300 mg Tablet should be discontinued.

Hyperammonaemia associated with neurological symptoms has also been reported. In such cases further investigations should he considered

Blood and lymphatic system disorders:

Frequent occurrence of thrombocytopenia. rare cases of anaemia, leucopenia or pancytopenia. The blood picture returned to normal when the drug was discontinued. Isolated reduction of fibrinogen or reversible increase in bleeding time have been reported, usually without associated clinical signs and particularly with high dose (sodium valproate has an inhibitory effect on the second phase of platelet aggregation). Spontaneous bruising or bleeding is an indication for withdrawal of medication pending investigation.

Skin and subcutaneous tissue disorders:

Cutaneous reactions such as exanthematous rash rarely occur with valproate. In very rare cases toxic epidermal necrolysis, Stevens-Johnson syndrome and erythema multiforme have been reported.

Transient hair loss, which may sometimes be dose related has often been reported. Regrowth normally begins within six months, although the hair may become more curly than previously. Hirsutism and acne have been vary rarely reported.

Reproductive system and breast disorders:

Amenorrhoea and irregular periods have been reported. Very rarely gynaecomastia has occurred.

Vascular disorders:

The occurrence of vasculitis has occasionally been reported.

Ear disorders:

Hearing loss, either reversible or irreversible has been reported rarely; however, a cause and effect relationship has not been established.

Renal and urinary disorders:

There have been isolated reports of a reversible Fanconi's syndrome (a defect in proximal renal tubular function giving rise to glycosuria, amino aciduria, phosphaturia, and uricosuria) associated with valproate therapy, but the mode of action is as vet unclear

Immune system disorders:

Allergic reactions (ranging from rash to hypersensitivity reactions) have been reported.

General disorders:

Very rare cases of non-severe peripheral oedema have been reported.

Increasing in weight may also occur. Weight gain being a risk factor for polycystic ovary syndrome. It should be carefully monitored.

DRUG INTERACTIONS

Effects of Valparin XR 300 mg Tablet on Other Drugs: Neuroleptics, MAOIs, Benzodiazepines and Antidepressants: Valparin XR 300 mg Tablet may potentiate the effect of other psychotropics eg, neuroleptics, MAOIs, benzodiazepines and antidepressants; therefore, clinical monitoring is advised and dosage should be adjusted when appropriate.

Phenobarbital: Valparin XR 300 mg Tablet increases phenobarbital plasma concentrations (due to inhibition of hepatic catabolism) and sedation may occur, particularly in children. Therefore, clinical monitoring is recommended throughout the first 15 days of combined treatment with immediate reduction of phenobarbital doses if sedation occurs and determination of phenobarbital plasma levels when appropriate.

Primidone: Valparin XR 300 mg Tablet increases primidone plasma levels with exacerbation of its adverse effects (eg, sedation); these signs cease with long-term treatment. Clinical monitoring is recommended especially at the beginning of combined therapy with dosage adjustment when appropriate.

Phenytoin: Valparin XR 300 mg Tablet decreases phenytoin total plasma concentration. Moreover, it increases phenytoin free form with possible overdosage symptoms (valproic acid displaces phenytoin from its plasma protein-binding sites and reduces its hepatic catabolism). Therefore, clinical monitoring is recommended; when phenytoin plasma levels are determined, the free form should be evaluated.

Carbamazepine: Clinical toxicity has been reported when Valparin XR 300 mg Tablet was administered with carbamazepine as Valparin XR 300 mg Tablet may potentiate toxic effect of carbamazepine. Clinical monitoring is recommended especially at the beginning of combined therapy with dosage adjustment when appropriate.

Lamotrigine: Valparin XR 300 mg Tablet may reduce lamotrigine metabolism and increase its mean half-life; dosage should be adjusted (lamotrigine dosage decreased) when appropriate. Co-administration of lamotrigine and Valparin XR 300 mg Tablet might increase the risk of rash.

Zidovudine: Valparin XR 300 mg Tablet may raise zidovudine plasma concentration leading to increased zidovudine toxicity.

Vitamin K-Dependent Anticoagulants: The anticoagulant effect of warfarin and other coumarin anticoagulants may be increased following displacement from plasma protein-binding sites by valproic acid. The prothrombin time should be closely monitored.

Temozolomide: Co-administration of temozolomide and Valparin XR 300 mg Tablet may cause a small decrease in the clearance of temozolomide that is not thought to be clinically relevant.

Effects of Other Drugs on Valproate: Antiepileptics with enzyme-inducing effect (including phenytoin, phenobarbital, carbamazepine) decrease valproic acid serum concentrations. Dosages should be adjusted according to blood levels in case of combined therapy. On the other hand, combination of felbamate and Valparin XR 300 mg Tablet may increase Valproate serum concentration. Valparin XR 300 mg Tablet dosage should be monitored.

In case of concomitant use of Valparin XR 300 mg Tablet and highly protein-bound agents (aspirin), free valproic acid plasma levels may be increased.

Valproic acid plasma may be increased (as a result of reduced hepatic metabolism) in case of concomitant use with cimetidine or erythromycin.

Mefloquine and chloroquine increases valproic acid metabolism and may lower the seizure threshold; therefore, epileptic seizures may occur in cases of combined therapy.

Accordingly, the dosage of Valparin XR 300 mg Tablet may need adjustment.

Carbapenem antibiotics such as imipenem and meropenem: Decrease in valproic acid blood level, sometimes associated with convulsion, has been observed when imipenem or meropenem were combined. If these antibiotics have to be administered, close monitoring of valproic acid levels is recommended. Cholestyramine may decrease the absorption of valproate.

Other Interactions: Caution is advised when using Valparin XR 300 mg Tablet in combination with newer antiepileptics whose pharmacodynamics may not be well established.

Valparin XR 300 mg Tablet usually has no enzyme-inducing effect; as a consequence, Valparin XR 300 mg Tablet does not reduce efficacy of oestroprogestative agents in women receiving hormonal contraception, including contraceptive pill.

Valproic acid may lead to an increased blood level of profolol. When co-administered with valproate, a reduction of the dose propofol should be considered.

DOSAGE AND ADMINISTRATION

Daily dosage requirements vary according to age and body weight.

In patients where adequate control has been achieved, Valparin XR 300 mg Tablet formulations are interchangeable with other conventional or prolonged-release formulations on an equivalent daily dosage basis

Epilepsy:

Monotherapy: Usual requirements are as follows:

Adults: Dosage should start at 600 mg daily increasing by 200 mg at 3-day intervals until control is achieved. This is generally within the dosage range of 1000-2000 mg/day ie, 20-30 mg/kg body weight. Where adequate control is not achieved within this range, the dose may be further increased to 2500 mg/day.

Children >20 kg: Initial dosage should be 400 mg/day (irrespective of weight) with spaced increases until control is achieved; this is usually within the range of 20-30 mg/kg body weight/day.

Where adequate control is not achieved within this range, the dose may be increased to 35 mg/kg body weight/day.

Elderly: Although the pharmacokinetics of valproate is modified in the elderly, they have limited clinical significance and dosage should be determined by seizure control. The volume of distribution is increased in the elderly and because of decreased binding to serum albumin, the proportion for free drug is increased. This will affect the clinical interpretation of plasma valproic acid levels.

Patients with Renal Insufficiency: It may be necessary to decrease dosage. Dosage should be adjusted according to clinical monitoring since monitoring of plasma concentration may be misleading.

Patients with Hepatic Insufficiency: Salicylates should not be used concomitantly with valproate since they employ the same metabolic pathway.

Liver dysfunction, including hepatic failure resulting in fatalities, has occurred in patients whose treatment included valproic acid.

Salicylates should not be used in children <16 years. In addition, in conjunction with Valparin XR 300 mg Tablet, concomitant use in children <3 years can increase the risk of liver toxicity.

Female children, female adolescents, women of childbearing potential and pregnant women

Valparin XR 300 mg Tablet should be initiated and supervised by a specialist experienced in the management of epilepsy. Treatment should only be initiated if other treatments are ineffective or not tolerated and the benefit and risk should be carefully reconsidered at regular treatment reviews. Preferably Valparin XR 300 mg Tablet should be prescribed as monotherapy and at the lowest effective dose, if possible as a prolonged release formulation to avoid high peak plasma concentrations. The daily dose should be divided into at least two single doses.

Female children and women of childbearing potential

Sodium valproate should not be used in female children and women of child bearing potential unless other treatments are ineffective or not tolerated (see Contraindications, Warnings and precautions and Fertility, pregnancy and lactation sections). The benefit and risk should be carefully reconsidered at regular treatment reviews. Sodium valproate should preferably be prescribed as monotherapy and at the lowest effective dose, if possible as a prolonged release formulation. The daily dose should be divided into at least two single doses.

Combined Therapy: When starting Valparin XR 300 mg Tablet in patients already
on other anticonvulsants, these should be tapered slowly; initiation of Valparin XR 300 mg Tablet therapy should then be gradual, with target dose being reached after about 2 weeks. In certain cases, it may be necessary to raise the dose by 5-10 mg/kg/day when used in combination with anticonvulsants which induce the liver enzyme activity eg, phenytoin, phenobarbitone and carbamazepine. Once known enzyme inducers have been withdrawn it may be possible to control seizure on a reduced dose of Valparin XR 300 mg Tablet. When barbiturates are being administered concomitantly and particularly if sedation is observed (particularly in children), the dosage of barbiturates should be reduced.

Note: In children requiring doses >40 mg/kg/day than clinical chemistry and haematological parameters should be monitored.

Optimum dosage is mainly determined by seizure control and routine measurement of plasma levels is unnecessary. However, a method for measurement of plasma levels is available and may be helpful where there is poor control or side effects are suspected.

Treatment and Prevention of Mania Associated with Bipolar Disorder: Adults: Recommended Initial Dose: 1000 mg/day. The dose should be increased as rapidly as possible to achieve the lowest dose, which produces the desired clinical
effect.

The recommended maintenance dosage for treatment of bipolar disorder is 1000-2000 mg daily. In exceptional cases, the dose may be increased to not more than 3000 mg daily. Doses should be adjusted according to individual clinical response.

Prophylactic treatment should be established individually with the lowest effective dose.

Administration

Should be taken with food. Swallow whole, do not chew/crush the tablet.

Mode of Administration: Oral

OVERDOSAGE

The clinical picture of massive acute intoxication usually includes coma, with hypotonia, decreased reflexes, myosis, and decreased respiratory function. Seizures have also been reported in presence of very high plasma levels.

The following measures should be implemented in hospitals: gastric lavage useful up to 10 to 12 hours following ingestion, institution of forced diureses, cardiorespiratory monitoring. In very severe cases, dialysis or exchange transfusion may be necessary. Naloxone has been successfully used in 1 case. Such intoxications usually have a good prognosis, though deaths have occurred following massive overdosage.

EXPIRY DATE:

Do not uses later than the expiry date.

STORAGE:

STORE BELOW 30°C. PROTECTED FROM MOISTURE.

Keep out of reach of children

PRESENTATION

VALPARIN XR 300 MG TABLET is available in Alu-Alu blister strip of 1x10, 3x10 & 10x10 tablets.

DATE OF REVISION

February 09, 2026

torrent PHARMA

Manufactured by :
TORRENT PHARMACEUTICALS LTD.
Indrad-382 721, Dist. Mehsana, INDIA.

PRODUCT NAME	:	VALPARIN XR 300	**COUNTRY :** Malaysia	**LOCATION :** Indrad		**Supersedes A/W No.:**	
ITEM / PACK	:	Insert	**NO. OF COLORS:** 1	**REMARK :**			
DESIGN STYLE	:		**PANTONE SHADE NOS.:**	**SUBSTRATE :**			
CODE	:	xxxxxxxxx-5253	Black	**Activities**	**Department**	**Name**	**Signature**	**Date**
DIMENSIONS (MM) (LxWxH) :		180 x 400		Prepared By	**Pkg.Dev**			
ART WORK SIZE	:	S/S		Reviewed By	**Pkg.Dev**			
DATE	:	09-02-2026		Approved By	**Quality**			